ADR REPORTING & DOCUMENTATION

Pharmacovigilance Internship – Task 2 | Completed Training Example

Educational-use notice
This is a fictional training case, not a real patient report. It must not be submitted to PvPI or any regulator as an actual safety report. Do not upload real patient identifiers to a public repository.

1. Objective

To practice structured documentation of a suspected adverse drug reaction (ADR), including patient details, reaction description, suspected medicine information, medical history, seriousness, outcome and reporter details. Complete documentation supports pharmacovigilance review and follow-up.

2. Fictional Case Scenario

A fictional 42-year-old adult was prescribed amoxicillin 500 mg orally every 8 hours for a suspected bacterial respiratory infection. On the third day of treatment, the patient developed an itchy red rash over the trunk and arms. The patient contacted a healthcare professional and was advised to stop the suspected medicine and seek assessment. The rash improved over the next few days. No hospitalization, life-threatening event or persistent disability was reported. Allergy history and the complete list of other medicines were not available in this teaching example.

3. A. Patient Information

Field	Sample entry
Patient code	Fictional Case A01 (not a real identifier)
Age	42 years
Sex	Not specified
Weight	Not available
Relevant medical history	Respiratory infection under treatment; other history unavailable
Known allergy history	Not available – follow-up information required

3. B. Suspected ADR Details

Field	Sample entry
Reaction description	Itchy red skin rash over trunk and arms
Onset	Day 3 of amoxicillin treatment; exact calendar date/time unavailable
End date	Not specified; improvement over following few days
Action taken	Healthcare professional advised stopping suspected medicine and seeking assessment
Treatment for reaction	Not specified
Outcome	Improving / recovering
Seriousness	Non-serious in this fictional example; no hospitalization, life-threatening event or disability reported

3. C. Suspected Medicine Details

Field	Sample entry
Suspected medicine	Amoxicillin
Strength / dose	500 mg

Field	Sample entry
Route / frequency	Oral / every 8 hours (as prescribed in this fictional case)
Indication	Suspected bacterial respiratory infection
Start date	Day 1 of fictional timeline; calendar date unavailable
Stop date	After rash onset, on healthcare professional's advice; exact date unavailable
Batch / expiry	Not available
Concomitant medicines	Not reported; must be verified during follow-up

3. D. Reporter Information

Field	Sample entry
Reporter name	Student training example – no real reporter
Role	Pharmacovigilance intern (educational exercise)
Contact details	Not applicable – fictional case
Report date	Not applicable – practice exercise

4. Timeline Summary

Time point	Event
Day 1	Amoxicillin treatment started.
Day 3	Itchy red rash appeared on trunk and arms.
After onset	Patient contacted a healthcare professional; advised to stop suspected medicine and seek assessment.
Following few days	Rash improved; exact resolution date unavailable.

5. Preliminary Assessment

Suspected drug: Amoxicillin.
Suspected reaction: Itchy red skin rash.
Reason for suspicion: The rash began after the medicine was started and improved after it was stopped on professional advice. This supports a possible association but does not prove causality.
Missing information: Exact dates, complete medical/allergy history, all concomitant medicines, clinical examination, treatment for rash and confirmation of full recovery.
Training causality assessment: Possible, based only on the limited fictional details. A real assessment requires complete case information and appropriate clinical review.

6. Documentation Quality Checklist

• Use a unique case code; do not expose patient name, phone number or address in a training file.

• Record the reaction clearly, with onset and outcome dates when known.

• Record medicine name, strength, dose, route, frequency, indication and start/stop dates.

• List concomitant medicines and relevant medical history, or mark unavailable information clearly.

• Assess seriousness using applicable criteria; do not equate symptom intensity with seriousness.

• Keep source information accurate, dated and traceable; update the report when follow-up information becomes available.

• For real suspected ADRs, use the current official reporting channel and follow institutional procedures.

7. Conclusion

This exercise demonstrates how a suspected ADR can be captured in a structured format. The sample documents patient profile, reaction, medicine details, chronology, outcome and reporter section while identifying missing information. Clear and

timely documentation supports medicine-safety review. This fictional exercise is for learning only and is not a substitute for an official ADR report.

8. References

1. Indian Pharmacopoeia Commission – Pharmacovigilance Programme of India (PvPI): https://ipc.gov.in/PvPI/
2. CDSCO – Central Drugs Standard Control Organisation: https://cdsco.mohfw.gov.in/
3. World Health Organization – Pharmacovigilance: https://www.who.int/teams/regulation-prequalification/regulation-and-safety/pharmacovigilance

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